

COPING WITH ADDICTION AND CHRONIC PAIN # 18

GOALS OF THE EXERCISE

1. Practice a program of recovery, including 12-step program participation and use of pain management skills.
2. Develop healthy options to cope with chronic pain.
3. Reduce daily suffering from pain and substance abuse.

ADDITIONAL HOMEWORK THAT CAN BE APPLICABLE TO CLIENTS WHO EXPERIENCE CHRONIC PAIN

- Alternative Methods for Managing Pain
- Alternatives to Addictive Behavior
- Assessing Self-Care Deficits
- Building My Support Network
- Coping with Addiction and Other Medical Problems
- Learning to Self-Soothe
- Physical and Emotional Self-Care
- Safe and Peaceful Place Meditation
- Use of Affirmations for Change
- Using My Support Network

SUGGESTIONS FOR PROCESSING THIS EXERCISE WITH THE CLIENT

The “Coping with Addiction and Chronic Pain” activity is designed for clients who, as the title indicates, suffer from both addictions and severe and persistent pain. It addresses the perceived dilemma many clients face of reconciling participation in 12-step recovery programs with the need to use prescribed medications that have a high potential for addiction, as well as noting other sources of emotional and practical support. Follow-up can include referral to appropriate medical professionals and to one or more of the chronic pain support groups cited in the exercise

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Some people suffer both from substance abuse problems and from other problems that cause them severe or chronic pain. This puts them in a dilemma, because the usual treatments for pain are narcotics or other strong medications, most of which are addictive. Often, the pain was what led these people to begin using addictive drugs to begin with—they may even have gotten addicted by using prescribed pain medications as instructed by their doctors. So if you are in this situation, how can you achieve and maintain quality of life, that is, a life free of both addiction and ongoing pain? You may have felt anxious, even hopeless, thinking about this problem. Fortunately, there are solutions. This exercise will help you understand the problem more completely and find some of those solutions.

1. First, from what kind of chronic pain do you suffer, and how does it affect your daily life?

2. Are you working with a doctor or other medical professional to manage your pain? _____ Is that person someone who knows about your problems with addiction and is experienced and qualified in working with people who suffer from addictions? _____

3. The risk in being treated by a professional who doesn't understand addictions is that he/she may prescribe a drug to which you are particularly vulnerable, may prescribe too much of a drug, or may not help you manage your use of pain medications well enough. Have you talked about these concerns with your medical provider? _____ What response did you get from him/her?

4. On the other hand, many medical professionals are reluctant to prescribe strong enough pain medications or to prescribe them at all, for fear of their patients getting addicted. Has your doctor talked about this with you? _____ If so, what did he/she say?

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5. Pain management is gaining recognition as a specific medical specialty. Are you working with a doctor who specializes in pain management? _____ If not, can the doctor or other professional with whom you're working give you a referral to a pain management specialist? _____ As part of this
6. In recent years the medical profession has found that even drugs that are otherwise highly addictive do not result in physical addiction if they are taken in appropriate amounts—that is, no more than is needed to control the pain. Studies have shown that patients receiving morphine and other narcotics did not develop physical dependence on those drugs and did not experience withdrawal when they stopped taking them as long as they didn't take too much of a drug and didn't keep taking it after the pain diminished to a point that could be managed with less powerful methods. Has your doctor talked with you about using medications this way? _____ If you do this, what is your plan to avoid taking more than you need and to stop taking the medications (probably by switching to something safer and not so strong) as soon as appropriate, to avoid becoming addicted?
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8. Another discovery that has changed the way pain medications are used: When the medication is taken before the pain gets severe, less of it is needed to block the pain and keep the patient comfortable. This is why hospitals now sometimes put people on a regular schedule for their pain medications and give them the medications even when they aren't too uncomfortable. Have you and your doctor talked about this, and if so, how do you feel about it?
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9. A danger to watch out for: many alcoholics and other addicts have found that when they used narcotics or equivalent drugs, although they didn't get addicted, their judgment and inhibitions were affected and they relapsed into drinking or using other drugs of choice, because they forgot why it was important for them to stay clean and sober. What is your plan to avoid this trap?
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10. The medical professionals have a central part to play in this kind of treatment, but other people also have important roles in helping you manage this situation (e.g., your sponsor if you're in a recovery program, family, friends). How can they help you avoid falling into addictive ways of thinking and behaving when you're using potentially habit-forming drugs to manage your pain?
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11. If you are participating in a 12-step recovery program, are you aware of the policies such programs have developed about the use of prescribed medications? What do you believe those programs have to say about this?
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Actually, the official position of Alcoholics Anonymous is that if your doctor knows you are an alcoholic or addict and has prescribed medication with that knowledge, and you are taking it exactly as prescribed, you are doing what you need to do to stay sober. Other programs have similar policies. If others in your 12-step group challenge this, they don't know their own program well enough. If you have questions, check the official literature.

12. Do you know others in a 12-step program who take powerful prescribed medications? How do they avoid falling into the trap of substance abuse?
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13. In addition to 12-step or other recovery programs that focus on addictions, you may find help from support groups specifically for chronic pain sufferers. These groups include many local organizations, which you may be able to find through your local media (many newspapers publish lists of support groups of all kinds). You may also want to investigate the following online groups, which you can reach through their web sites (please keep in mind that as time passes, these sites may disappear and others appear—to get current information, we recommend using an Internet search engine such as Google.com).

- a. American Chronic Pain Association: www.theacpa.org
- b. Back Pain Support Group: www.backpainsupportgroup.com
- c. Chronic Pain Support Group: www.chronicpainsupport.org
- d. National Chronic Pain Outreach Association, Inc.: www.chronicpain.org
- e. Out of Pain: www.outofpain.com

14. Have you had any contact with any of these groups, either local or online? _____ If so, please identify the groups and briefly write about your experiences with them.
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15. Please use this space to describe the tools, methods, and resources you will use to cope with the combined challenges of an addiction and chronic pain.
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